

Trust Policy		
Moving and Handling People and Objects		
Issue Date	Review Date	Version
July 2021	July 2026	3
Purpose		
This policy is intended to support the organisation, in reducing the risk of Moving and Handling injuries by providing guidance on the measures that should be taken, to ensure safe moving and handling can be applied in the workplace.		
Who should read this document?		
• Trust Board and its committees, because it is ultimately the Board's responsibility and accountability to ensure that relevant Health and Safety legislation is being addressed. • Senior clinicians and senior managers, because they need to know their responsibilities and accountability in respect of promoting safe handling behaviours in their work areas • All staff, because they need to know their responsibilities and accountability and what is expected of them in terms of safe handling; and where they should go to get training, advice and support in addressing manual handling issues.		
Key Messages		
Reduce the risk of moving and handling as far as is reasonably practical by following safe systems of work. These include ensuring staff are provided with information, instruction, training and supervision. Staff must be appropriately trained in carrying out moving and handling activities including the use of appropriate manual handling equipment. • All moving and handling risks must be identified and appropriate action taken to mitigate incidents of avoidable harm. • All moving and handling incidences must be reported and acted upon, including lessons learned and identified to relevant teams. • All staff are responsible for ensuring that they are adequately trained in moving and handling techniques. • The Moving and Handling Team Specialist Advisors will promote the sharing of good practice and learning from experience and will act as an advisory service to assist the Trust in meeting its responsibilities. • New moving and handling equipment must only be introduced to the Trust through the correct approval and procurement process. • Staff must apply safe, dignified approaches when moving and handling patients.		

Core accountabilities		
Owner		Moving and Handling Lead
Review		Health and Safety Committee
Ratification		Deputy Chief Nurse
Dissemination (Raising Awareness)		Moving and handling lead
Compliance		Health and Safety Committee
Links to other policies and procedures		
Health and Safety Policies and Procedures Trust Documents Risk Management Framework Incident Management Policy Risk and Incident Policy Root Cause Analysis (RCA) for Adverse Events Investigation Plus size moving and Handling Policy Moving and Handling SOP		
Version History		
	March 2003	Approved by the Health & Safety Committee
	September 2008	Approved by the Health & Safety Committee
	March 2010	Approved by the Health & Safety Committee
	August 2011	Revision of document to reflect NHSLA expectations.
1	March 2012	Revision of document to reflect organisational responsibilities and NHSLA expectations
2	February 2015	Changed from SOP to Policy and Approved by the Health & Safety Committee. Ratified by Director of Corporate Business
2.1	March 2020	Extended to May 2020 by Lee Budge
2.2	February 2021	Extended to May 2021
3	July 2021	Updated and approved by Health & Safety Committee

The Trust is committed to creating a fully inclusive and accessible service. Making equality and diversity an integral part of the business will enable us to enhance the services we deliver and better meet the needs of patients and staff. We will treat people with dignity and respect, promote equality and diversity and eliminate all forms of discrimination, regardless of (but not limited to) age, disability, gender reassignment, race, religion or belief, sex, sexual orientation, marriage/civil partnership and pregnancy/maternity.

An electronic version of this document is available on Trust Documents. Larger text, Braille and Audio versions can be made available upon request.

Contents

Section	Description	Page
1	Introduction	4
2	Purpose, including legal or regulatory background	4
3	Definitions	5
4	Duties	5-10
5	Formal document Development	10-12
6	Overall Responsibility for the Document	12
7	Consultation and Ratification	12
8	Dissemination and Implementation	13
9	Monitoring Compliance and Effectiveness	13-14
10	References and Associated Documentation	14
Appendix 1	Dissemination Plan and Review Checklist	15
Appendix 2	Equality Impact Assessment	16
Appendix 3	Manual Handling – Organisational Advice Structure	17
Appendix 4	Moving and Handling Risk Assessment & Care Plan Guidance	18-19
Appendix 5	Paediatric manual Handling mobility assessment guidance	20-21

1 Introduction

Musculoskeletal (MSK) disorders can include back pain, neck or arm strains and diseases of the joints. All parts of the workforce can be affected by MSK. It is one of the most common reasons for sickness absence in the NHS.

It accounts for 40 per cent of all sickness absence which in many cases become long-term absences. MSK may be caused by work-related injuries, such as incorrect handling, or not having equipment in the workplace personally adjusted. Prevention and early intervention is key to effectively managing MSK disorders (NHS employers 2019) .

Under Section 2 of the Health and Safety at Work Act 1974, every employer has the duty to provide a safe place of work, a safe environment and safe systems of work, so far as is reasonably practicable. This duty includes the need to minimise risk arising from moving and handling tasks.

Under the Manual Handling Operations Regulations 1992 (MOHR), moving and handling is interpreted as the transporting or supporting of any load.

Regulation 4 of MHOR requires the employer to avoid the need for hazardous moving and handling activities, so far as is reasonably practicable. Where it is not possible to eliminate hazardous moving and handling, an assessment must be undertaken to determine the level of risk. Suitable controls must then be introduced to reduce the risk of injury to the lowest extent reasonably practicable.

The application of ergonomic principles and structured organisational arrangements can effectively reduce the existing level of personal injury arising from moving and handling activities at work. Tasks need to be assessed and modified as necessary to reduce the risk and incidence of injury and cumulative trauma.

2 Purpose

This document describes the responsibilities for all University Hospital Plymouth NHS Trust staff grades with regard to safe handling practice.

The Trust has a commitment to eliminate hazardous moving and handling as far as is reasonably practicable. As an employer, it is bound by the Health and Safety at Work Act, 1974. The Manual Handling Operation Regulations (1992) give further explicit and legislative information regarding duty of care. The emphasis on the regulations is on avoidance of moving and handling, and where there is a risk of injury if this cannot be avoided, a full moving and handling risk assessment must be conducted to help reduce the risk to a reasonable practical level.

This procedure sets out the staff responsibilities in relation to the Trust's intentions, as far as is practicable, to:

- Avoid the need for hazardous moving and handling operations.
- Provide safe and suitable workplace environments and working practices.
- Assess the risk of injury from any hazardous moving and handling that can't be avoided and introduce arrangements to reduce these to the lowest level practicable.
- Provide equipment to enable moving and handling activities to be undertaken safely.
- Provide suitable and sufficient supervision and training to staff involved in moving and handling operations. Reassessments may also be required where accident/absence statistics show that the original control measures were not sufficiently effective.

3 Definitions

Moving and Handling - the transporting or supporting of loads, by human effort which includes any lifting, putting down, pushing, pulling, carrying or supporting a load by hand or by bodily force.

Moving and Handling injury – physical damage of an individual, occurring as a result of a one-off incident, or from a build-up of trauma over time. Includes injury to patients occurring as a result of a moving and handling incident.

Risk – the chance that an event will occur that will impact adversely on the Trust's objectives.

Hazard – a condition that gives rise to, or increases the risk of an adverse event occurring.

Likelihood – a measure of the probability that the predicted event will occur.

Consequence – a measure of the adverse impact of the predicted event, in terms of harm, loss or damage on people, property, or the Trust's objectives.

Incident – for the purposes of this procedure, is any unwelcome outcome arising from a moving and handling action, regardless of the level of injury resulting.

4 Duties

4.1 Trust Board and Chief Executive

4.1.1 The Board retains overall responsibility for the Health Safety and welfare of employees and non-employees. Responsibility for gaining assurance that these statutory responsibilities are delivered is delegated to the Director or Governance. The Chief Executive is ultimately responsible for ensuring that the Trust maintains adequate procedures for ensuring that, as far as practicable, patients and staff are kept safe from the risk of harm from incidents arising from moving and handling activities.

4.2 Deputy Director of Nursing

- 4.2.1 Under delegated authorities, the Deputy Director of Nursing is responsible for gaining assurance that adequate arrangements are in place and maintained to manage and mitigate the risk of injury arising from moving and handling activities. This responsibility is delivered through:
- 4.2.2 Monitoring reports from the Health and Safety Committee on the health and safety performance of the Trust in relation to moving and handling.
- 4.2.3 Health and Safety Committee report direct to the people and Culture Committee.
- 4.2.4 Ensuring that sufficient resources are made available to enable the Moving and handling Team to provide an adequate overview and assurance of moving and handling risk across the Trust.

4.3 Health and Safety Committee

4.3.1 Health and Safety Committee report directly to the Trust board. They are responsible for ensuring that the Trust maintains adequate arrangements to manage and mitigate the risks of injury arising from moving and handling activity. This responsibility is delivered through:

- 4.3.2 Receipt and consideration of periodic and exception reports from the Trust's Health and Safety Committee, covering reporting of specific incidents, complaints and claims, or significant risks identified. This will include:
- Analysis of trends in risk assessments, incidents, complaints and claims specific to moving and handling.
 - Any aspects of Manual Handling that the Group deems need to be reported.

- 4.3.3 Review and approval of procedural documents relating to moving and handling. This responsibility may be delegated formally to staff groups with specialist knowledge.

4.4 Service Line Directors, Cluster Managers, Heads of Departments and Matrons

- 4.4.1 Ensure the day to day health, safety and welfare of all staff, patients and members of the public in areas of their control.
- 4.4.2 Ensure that where moving and handling risks are identified, they are supported with the relevant action plans, reviewed and evaluated to reduce the risk of injury to staff and patients.

4.5 The Director of Nursing

- 4.5.1 Ensures that arrangements are in place to take account of nationally and locally agreed moving and handling guidance so that moving and handling best practice is employed in the context of clinical services provided by the Trust.

4.6 Clinical Engineering is responsible for:-

- 4.6.1 Ensuring that all moving and handling equipment including attachments is maintained in accordance with the Lifting Operations and Lifting Equipment Regulations (LOLER 1998) ensuring 6 monthly inspections are carried out and recorded.

4.7 Moving and Handling Lead

- 4.7.1 Provides strategic advice and support, Trust wide, to facilitate an effective programme of risk management of moving and handling activities, including advising on the development of action plans to address risks identified. Provide specific training and advice as appropriate.
- 4.7.2 Lead the Moving and Handling Team who are the Trust's specialist advisors on moving and handling issues (see Appendix 3).
- 4.7.3 Manage the Moving and Handling Team in advising on moving and handling issues identified on the risk register, to develop focus groups and devise and implement relevant action plans to mitigate risks to the Trust.
- 4.7.4 Lead the team in the design and delivery of mandatory moving and handling training, including induction for both clinical and non-clinical staff Trust wide.
- 4.7.5 Lead the team on plus size management Trust wide contacting infection control, tissue viability and ergonomics as required.
- 4.7.6 Monitor compliance and effectiveness of this policy through audit process to provide an organisational overview of moving and handling.

- 4.7.7 Design audit programmes, as necessary, to facilitate the organisation to benchmark their own moving and handling arrangements and services in line with agreed moving and handling procedures.
- 4.7.8 Leads/advises on liaising with suppliers/manufacturers of equipment to develop equipment identified for the needs of the Trust.
- 4.7.9 Produce monthly reports findings that will be presented to the Health and Safety Committee in accordance with their terms of reference
- 4.7.10 Lead the team in maintaining a portfolio of procedures and guidance, for staff, on safe handling of patients and inanimate loads. Include procedures for the operation of equipment, such as hoists, beds and trolleys. Include procedures for handling difficult loads, including specific patient groups. Safe operative procedure for manual handling Techniques can be accessed Trust documents. All wards and departments are required to have a copy available for staff to view. Lead the team in reviewing moving and handling techniques as appropriate based on risk.
- 4.7.11 Lead on design delivery and evaluation of Moving and Handling Key worker course content ensuring programme comes in line with National Back Exchange Guidance.

4.8 Moving and Handling Team

- 4.8.1 Provide specialist advice and support Trust wide in completing moving and handling risk assessments. Provide advice on safe handling practices and use of specialised equipment.
- 4.8.2 Provide advice and support to Moving and Handling Key Workers at ward/department level.
- 4.8.3 Monitor incidents, complaints and claims, and significant risks identified and reported in the Trust's (Datix) risk assurance database. Notify Patient Safety Team of any significant or serious events.
- 4.8.4 Provide Trust wide specialist advice on complex moving and handling issues including plus size patients.
- 4.8.5 Conduct audits of handling practices and support moving and handling key workers in monitoring practice and local level.
- 4.8.6 Designs, delivers and evaluates Trust wide moving and handling programmes.
- 4.8.7 Delivers and evaluates moving and handling key worker programme in line with National Back Exchange guidance.

4.9 Moving and Handling Key Worker

- 4.9.1 Act as a local point of reference, supporting and passing on basic knowledge and skills to colleagues.
- 4.9.2 Seek specialist advice, as required, regarding safe moving and handling issues from the Moving and Handling Team.
- 4.9.3 Support the Local manager in reviewing local moving and handling equipment ensuring mechanical aid LOLER checks are in date.
- 4.9.4 Attend key worker update course, annually to maintain competence.
- 4.9.5 Assessors must have been assessed as competent to practice the indicated skill by the Moving and Handling Facilitator.

- 4.9.6 Assess core skills of staff within their area for moving and handling updates and if necessary deliver training e.g. unsuccessful assessment.
- 4.9.7 Each assessment must be fully completed, signed and dated by the individual and the assessor.
- 4.9.8 This information must then be scanned and submitted by email to the workforce development mailbox, where it will be recorded centrally on the Trust system.
- 4.9.9 Ensure that the signed competence assessment paperwork is stored for evidence and maintained locally in line with all HR records and retained for 6 years after the individual leaves service. At which time a summary of the file must be kept until the individual 70 birthday, a paper copy can be scanned and retained.
- 4.9.10 The core moving and handling skills will be assessed minimum 2 yearly.
- 4.9.11 If any of the performance criteria are failed on a task, the key worker must complete an action plan stating the intervention, by whom and completion date.
- 4.9.12 Following intervention from the first assessment, a second assessment must be undertaken (where appropriate by a different assessor) as soon as possible.
- 4.9.13 If any of the performance criteria on a task are failed twice, a third assessment may be undertaken by the moving and handling team, and findings reported to the line manager. Any employees failing to achieve competency after three assessments, will be deemed unsuccessful and referred back to their manager for further action.
- 4.9.14 Assist the manager in the undertaking of moving and handling risk assessments/audits as applicable.
- 4.9.15 Provide local moving and handling training on mechanical aids and new equipment as appropriate, as part of local induction.
- 4.9.16 Maintain a moving and handling file that includes local written documentation such as local moving and handling competencies, specific 'in house' training and mechanical aid training.

4.10 Departmental and Ward Managers

- 4.10.1 Ensure that all staff, including temporary staff e.g. NHSP are aware of the moving and handling procedures and have received up to date training.
- 4.10.2 Carry out planned periodical assessment of moving and handling risks in their working environment and record the results in line with the Trust's risk management policy.
- 4.10.3 Ensure that individual patient handling risk assessments are completed for all patients within 6 hours of admission using the Individual Patient Handling Risk Assessment and Care Plan.
- 4.10.4 Ensure risks identified on the risk register are supported with relevant action plans, that delivery of the required actions is monitored and further actions identified if needed
- 4.10.5 Address hazards identified related to moving and handling. Ensure that immediate actions are taken to reduce the impact of the hazard and assess any residual risk, recording findings in the Trust's (Datix) risk database, in accordance with the Trust's risk management policy.
- 4.10.6 Review all reports of incidents related to moving and handling practice and ensure that an action plan identifying the remedial measures required is in place.
- 4.10.7 Contact Moving and Handling Team for specialist advice when required on Tel. 39054 or bleep 81413 for urgent advice.

- 4.10.8 Report malfunctioning moving and handling aids, such as hoists, beds and trolleys to Clinical Engineering at the earliest opportunity, take them out of service and seek the appropriate expert advice before allowing them to be used.
- 4.10.9 Nominate moving and handling key workers for each department, or ward area to conduct moving and handling risk assessments and provide onsite training, assessment of competencies and monitoring of practice. Design working arrangements that enable the moving and handling key worker to deliver this additional role.
- 4.10.10 Ensure that staff who are absent from work with musculoskeletal injury are referred to the Occupational Health and Wellbeing Department for assessment and guidance regarding returning to work and changes in working arrangements made as appropriate.
- 4.10.11 Ensure a risk assessment is conducted for new and expectant mothers in line with Trust policy.

4.11 All staff

- 4.11.1 All staff are expected to take practical steps to minimise risk to themselves, patients and colleagues. Wherever there is doubt or uncertainty, staff are expected to seek assistance necessary to make the procedure safer. All staff are responsible to:
- 4.11.2 Report all moving and handling incidents, in line with the Trust's Adverse Events policy.
- 4.11.3 All registered nursing staff to ensure that individual patient handling risk assessments are completed for all patients within 6 hours of admission and are updated if the patient's condition changes, or within 3 days. (Whichever is the earliest).
- 4.11.4 Understand the Trust's procedural documents on managing moving and handling risks and ensure that they have an up to date record of completion of required training, as detailed in the Trust's Workforce Induction and Training Policy.
- 4.11.5 Use only equipment and procedures for which they are trained and competent to use.
- 4.11.6 Seek basic guidance from the moving and handling key worker, or the moving and handling team for specialist advice, for any situations where they are uncertain of the best approach to use.
- 4.11.7 Report discomfort or pain possibly associated with their work to their line managers and refer to the Staff to Occupational Health and Wellbeing department as appropriate in line with Trust policy
- 4.11.8 Ensure that they do not create hazards or increase risks as a result of their working practices and behaviours

4.12 Therapeutic Handlers

4.12.1 Therapeutic handling is balancing the potential benefits to patients arising from therapeutic intervention, involving moving and handling risk to themselves, the patient and colleagues.

The intervention/treatment involving moving and handling and the development of skills in order to carry out such interventions are essential core elements of practice.

Prior to any therapy task which specifically involves moving and handling a therapeutic risk assessment should be completed.

This will involve the following considerations:

- The task the patient is to achieve
- The patient's ability
- The clinical reasoning
- Alternatives
- Patient's position
- The therapist's role
- Modifications
- Hazards / risks

Risk reducing strategies need to be explored such as the need for extra equipment, additional staff, skills and competence, and the need for management commitment to reduce risks to both staff and patients when conducting therapeutic interventions.

5 General Moving and Handling responsibilities

Risk Assessment

5.1 Moving and Handling Risk Assessments for moving and handling of patients and objects.

5.1.1 Manual Handling Operations which involve a risk of injury should, where possible, be avoided. The risk of injury related to any remaining manual handling operations, or where the task cannot be avoided, must be reduced to the lowest level reasonably practicable. A basic assessment should consider:

- Load
- Individual capability
- Task
- Environment
- Other factors such as personal protective equipment, level of experience and individual concerns should also be considered.

5.1.2 Moving and Handling risk assessments must be carried out in accordance with the Risk Management Policy & Procedure for Managing Risks which can be found on Trust documents

5.1.3 All areas should have a generic overarching moving and handling risk assessment for their department entered onto Datix that is reviewed annually, or more frequently dependent on the actions identified. Actions will be identified, monitored, reviewed and updated by the manager and supported for initial advice by the moving and

handling key worker. For expert moving and handling advice contact the Moving and Handling Team bleep: 81413.

- 5.1.4 Task specific moving and handling Hazards can be identified using the MAC or RAPP for pushing and pulling of loads
- 5.1.5 The risk assessment is entered onto Datix with actions and review dates identified. Actions will be monitored, reviewed in line with Risk Management Policy & Procedure for Managing Risks.
- 5.1.6 An Individual Moving and Handling Risk Assessment and care plan must be completed for all patients who require assistance to move (See appendix 4)
- 5.1.7 Managers and Key workers can seek specialist advice in relation to moving and handling risk assessments from the Moving and Handling Team.
- 5.1.8 Organisational overview of compliance for managing of risks relating to the moving and handling of patients and objects will be conducted by Moving and Handling Team on an annual basis and appropriate recommendations and actions will be reported to service line and departmental managers and the Health and Safety Committee who has oversight

5.2 Moving and Handling Training.

- 5.2.1 Moving and handling training is a statutory requirement. All Clinical and Non clinical staff will attend face to face moving and handling induction within 4 weeks of employment with the exception of “new to Care “staff or International nurses. These staff will receive training prior to commencement in clinical practice. Videos of Safe Handling techniques are located on Trust net and SOP are stored in G drive in Trust Documents. They are also held locally in department’s resource folder.
- 5.2.2 Department of Professional Health Care Education record the attendance of Mandatory Moving and Handling Induction Training on OLM which is a centrally held Data base.
- 5.2.3 Compliance will be monitored by the Organisational Development Department. DNA’s will be recorded via the workforce dash board. Service line managers will be responsible for ensuring all staff complete moving and handling induction in line with Policy.

Moving and Handling Mandatory Update Competencies

- 5.2.4 Mandatory update training will be assessed in the work place minimum 2 yearly, by the MH Key workers for both Clinical and non-clinical areas.
- 9.1.2 The mandatory training competencies for all clinical and non - clinical staff will be completed as part of the blended learning approach, and work in conjunction with yearly on line learning and practical videos.
- 5.2.6 Mandatory MH competencies records will be maintained by the Workforce Development. Compliance will be monitored by the Organisational Development Department feeding back to service line managers via the Dashboard.
- 5.2.7 When new moving and handling equipment is purchased, procurement will arrange for the manufacturer to provide training to all staff in that area. This will include more in-depth training for the Moving and Handling Team and MH key workers to enable them to deliver future training to staff in relation to its use.

5.3 Equipment

- 5.3.1 All handling aids should be supplied and maintained in accordance with the Lifting Operations and Lifting Equipment Regulations and the Provision and Use of Work Equipment Regulations. Refer to moving and handling resource folder and Medical Equipment Maintenance Service.
- 5.3.2 All new equipment purchased must be approved by the Moving and Handling Team. Medical equipment maintenance service must be informed of purchase of mechanical aids.
- 5.3.3 The maintenance and repair of mechanical handling equipment will be coordinated by the Clinical Engineering Department.
- 5.3.4 Guidelines for laundering/cleaning of equipment can be found in the manufactures instructions held in the moving and handling resource folder for further guidance refer to Control of Infection Manual.

5.4 Specific Arrangements for New and Expectant Mothers

- 5.4.1 As soon as the HR admin team is informed of a pregnancy, they will send the relevant departmental or ward manager the risk assessment for new, expectant and breastfeeding mothers to complete, if they have not already done so. The manager should agree provisions with the staff member for reducing moving and handling, as necessary. The manager should contact HR Direct for advice if they are not able to accommodate these provisions.
- 5.4.2 Where the manager is made aware of a pregnancy, before it is formally reported to the Trust, the assessment should still be completed and changes in working arrangements made as appropriate.

6 Overall Responsibility for the Document

The moving and handling lead working in conjunction with the Deputy Head of Nursing and the Health and Safety committee have responsibility for developing, implementing and reviewing this policy.

7 Consultation and Ratification

The design and process of review and revision of this policy will comply with The Development and Management of Formal Documents.

The review period for this document is set as default of five years from the date it was last ratified, or earlier if developments within or external to the Trust indicate the need for a significant revision to the procedures described.

This document will be reviewed by the Health and Safety Committee and ratified by the Deputy Head of Nursing.

Non-significant amendments to this document may be made, under delegated authority from the Deputy Head of Nursing, by the nominated owner. These must be ratified by the Deputy Director of Nursing.

Significant reviews and revisions to this document will include a consultation with named groups, or grades across the Trust. For non-significant amendments, informal consultation will be restricted to named groups, or grades who are directly affected by the proposed changes.

8 Dissemination and Implementation

Following approval and ratification, this policy will be published in the Trust's formal documents library and all staff will be notified through the Trust's normal notification process.

Document control arrangements will be in accordance with The Development and Management of Formal Documents.

The document owner will be responsible for agreeing the training requirements associated with the newly ratified document with the Deputy Head of Nursing and for working with the Trust's training function, if required, to arrange for the required training to be delivered.

9 Monitoring Compliance and Effectiveness

- 9.1 The purpose of monitoring will be to assess compliance with this policy and to identify any current issues such as lack of equipment, risk assessments and staff training needs etc. with an overall objective of creating a plan to proactively support the managers and staff to improve practice in their area.
- 9.1.1 A Trust wide audit with particular attention to identify high risk areas will be co-ordinated by the moving and handling team on a yearly basis. This will focus on the following areas of compliance:
- 9.1.2 Overarching moving and handling risk assessments are completed in date with appropriate action plans.
- 9.1.2 Task specific risk assessments as appropriate are completed in date with appropriate action plans.
- 9.1.4 Areas have active key workers
- 9.1.5 A report of findings will be presented to the Health and Safety committee by the Moving and Handling Lead, and disseminated to matrons and ward managers.
- 9.1.6 Audits will be conducted in non-clinical areas and findings presented to the Health and Safety committee disseminating to findings to managers.
- 9.1.7 The availability of patient handling equipment will be monitored by the moving and handling key worker at local level.
- 9.1.8 Service Line managers will monitor the level of sickness absence related to moving and handling and investigate areas where levels of injury or cumulative trauma are above average for University Hospitals Plymouth
- 9.1.9 Audit of practice in specific areas may be conducted following reports from staff during training and on review of reported incidents.
- 9.1.10 Individual Patient handling risk assessment and care plan will be monitored by the moving and handling team using Meridian. Verbal reports will be given at time of audit. All managers and matrons are able to access this live tool reports will be sent to matrons and managers who are responsible for delivery of arising actions.
- 9.1.11 All moving and handling incidents must be investigated by the department/ward manager and a risk assessment conducted. Following investigation, the ward/department manager must develop a remedial action plan and monitor the progress against set time-scales providing feedback to the service line manager when difficulties with implementation arise.
- 9.1.12 The Moving and Handling Team monitor and review moving and handling incidents to establish an overview of key themes and trends; this is reported via the health

and safety dash board to the health and safety committee by the Moving and Handling Lead.

9.1.13 The workforce development team within the Human Resources and organisational development department will monitor training records and produce monitoring reports on a monthly basis for the Associate Director of Human Resources and Organisational Development to take to the Trust Board.

9.1.14 On a monthly basis, statistics regarding moving and handling are reported to the Health and Safety Committee in relation to incidents and key issues. Where areas for learning and improvement are identified, an action plan will be agreed and implemented with progress reported to and monitored by the health and safety committee.

9.1.15 The Board will receive reports from the health and safety committee.

10 References and Associated Documentation

Manual Handling Operations Regulations (1992) regulation 4, as amended by the Health and Safety Executive (Miscellaneous Amendments) Regulations (2002) Manual Handling Operations Regulations 1992.HMSO. London.

Management of Health and Safety at Work Regulations. (1999) HMSO: London

Health and Safety at Work Act (1974) HMSO: London.

The Guide to the Handling of People. (2011) Middlesex: Back Care. RCN. National Back Exchange

The Health and Social Care Act (2008) (Regulated Activities) Regulations 2014 HMSO:London

Specific guidance on Manual Handling in physiotherapy (4th edition) 2014

The following documents are referred to in this policy, or provide additional sources of reference material:

NHSLA Risk Management standards 2013-14

Further information: The following is a list of further information and guidance relevant to manual handling:

Back Care, RCN, 2005, The Guide to the Handling of Patients, 5th Edition, Ed. Smith J., Back Care (NBPA), Middlesex.

HSE, Manual Handling Operations Regulations 1992 (as amended) Guidance on Regulations, L23, 2016

HSE, 1998 Lifting Operations and Lifting Equipment Regulations 1998. Approved Code of Practice and Guidance L113, HSE Books

HSE, Simple Guide to the Provision and Use of Work Equipment Regulations 1998

HSE (2007) Risk assessment and process planning for bariatric patient handling pathways

Hignett, S., 2005, Measuring the Effectiveness of Competency Based Education and Training Programmes in Changing the Manual Handling Behaviour of Healthcare Staff. HSE Books Research Report 315

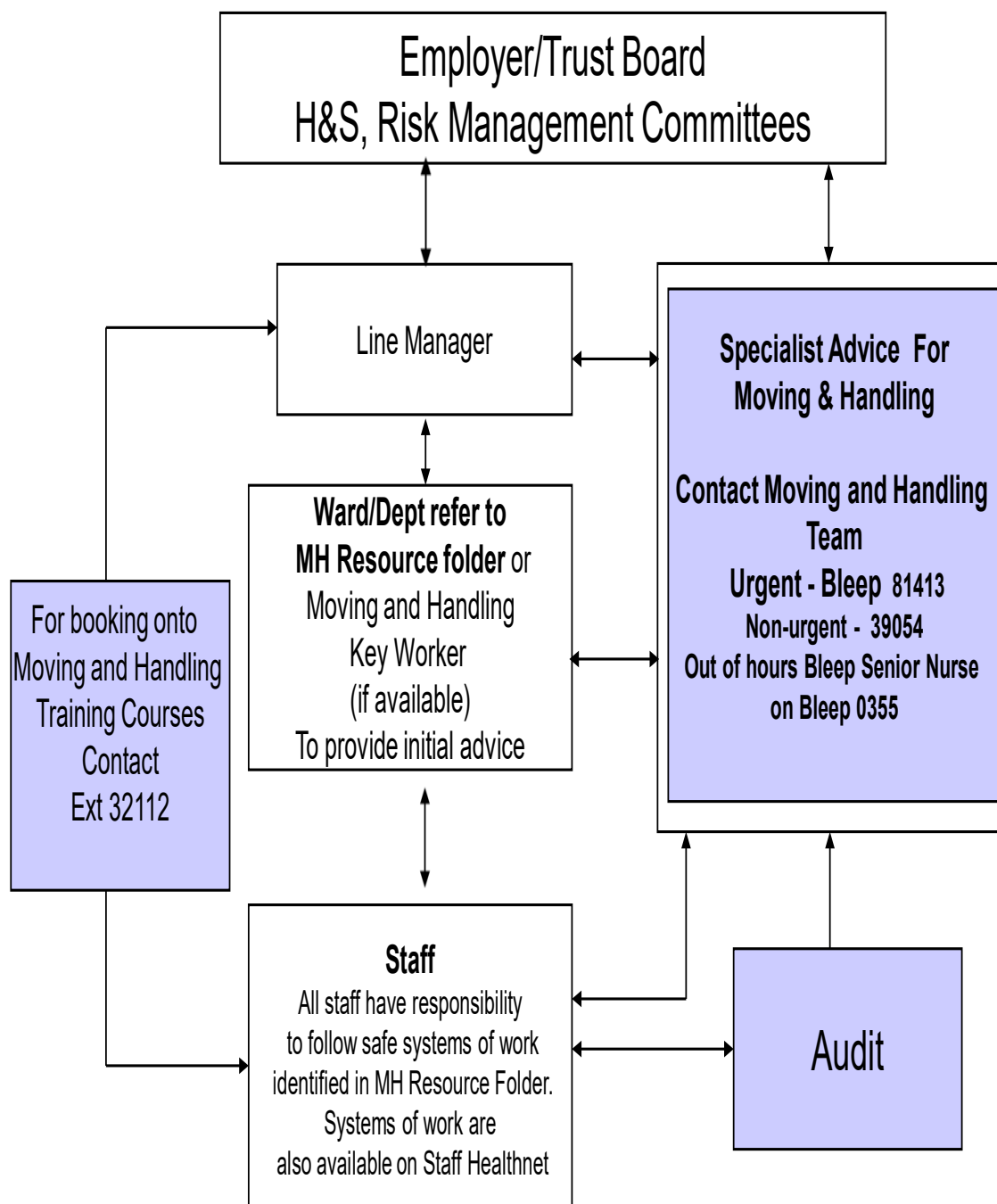
RCN, 2003, Safer Staff, Better Care, RCN Manual Handling Training Guidance and Competencies, RCN, London

Smith, J. (ed.) (2011) The Guide to the Handling of People, 6th edn. Teddington: Backcare

Dissemination Plan and Review Checklist			Appendix 1	
Dissemination Plan				
Document Title		Moving and Handling People and Objects policy		
Date Finalised		28/7/2021		
Previous Documents				
Action to retrieve old copies		Removed from Trust document folder		
Dissemination Plan				
Recipient(s)		When	How	Responsibility
All Trust staff		July 2021	Information Governance StaffNet Page	Information Governance Team
Review Checklist				
Title	Is the title clear and unambiguous?			Y
	Is it clear whether the document is a policy, procedure, protocol, framework, APN or SOP?			Y
	Does the style & format comply?			Y
Rationale	Are reasons for development of the document stated?			Y
Development Process	Is the method described in brief?			Y
	Are people involved in the development identified?			Y
	Has a reasonable attempt has been made to ensure relevant expertise has been used?			Y
	Is there evidence of consultation with stakeholders and users?			Y
Content	Is the objective of the document clear?			Y
	Is the target population clear and unambiguous?			Y
	Are the intended outcomes described?			Y
	Are the statements clear and unambiguous?			Y
Evidence Base	Is the type of evidence to support the document identified explicitly?			Y
	Are key references cited and in full?			Y
	Are supporting documents referenced?			Y
Approval	Does the document identify which committee/group will review it?			Y
	If appropriate have the joint Human Resources/staff side committee (or equivalent) approved the document?			Y
	Does the document identify which Executive Director will ratify it?			Y
Dissemination & Implementation	Is there an outline/plan to identify how this will be done?			Y
	Does the plan include the necessary training/support to ensure compliance?			Y
Document Control	Does the document identify where it will be held?			Y
	Have archiving arrangements for superseded documents been addressed?			Y
Monitoring Compliance & Effectiveness	Are there measurable standards or KPIs to support the monitoring of compliance with and effectiveness of the document?			Y
	Is there a plan to review or audit compliance with the document?			Y
Review Date	Is the review date identified?			Y
	Is the frequency of review identified? If so is it acceptable?			Y
Overall Responsibility	Is it clear who will be responsible for co-ordinating the dissemination, implementation and review of the document?			Y

Equalities and Human Rights Impact Assessment				Appendix 2	
Core Information					
Date	July 2021				
Title	Moving and Handling People and Objects Policy				
What are the aims, objectives & projected outcomes?	To comply with the statutory requirement to embed safe practice in moving and handling for all staff patients across all sites.				
Scope of the assessment					
Collecting data					
Race	No				
Religion	No				
Disability	No				
Sex	No				
Gender Identity	No				
Sexual Orientation	No				
Age	No				
Socio-Economic	No				
Human Rights	No				
What are the overall trends/patterns in the above data?	NA				
Specific issues and data gaps that may need to be addressed through consultation or further research	No Data was Collated during this review				
Involving and consulting stakeholders					
Internal involvement and consultation	Trust wide clinical & Non clinical staff to encompass multidisciplinary Groups of staff including managers and matrons. Members of the Trust's Health and Safety Committee including JSNC representative, Security Management Specialist, Director of Corporate Business, Staff Health and Wellbeing and Trust Specialist Advisors				
External involvement and consultation	NA				
Impact Assessment					
Overall assessment and analysis of the evidence	This document provides a comprehensive policy which encourages, endorses and guides all persons to reduce the risk of Moving and Handling in a way that promotes a safe and embeds safe practice for all staff, patients.				
Action Plan					
Action	Owner	Risks	Completion Date	Progress update	

Manual Handling - Organisational Advice Structure



Moving and Handling Risk Assessment and Care Plan Guidance

A. Standard

An Individual Moving and Handling Risk Assessment must be completed for all patients. Patients who require assistance with their mobility must have a Moving and Handling Care plan (Manual Handling Operation Regulations 1992). i.e. completion of this form is a statutory requirement.

B. General information regarding assessment contained within the Risk Assessment Booklet:

1. All patients admitted must have a risk assessment completed within 6 hours of admission. The Assessment and Care plan may also be used within outpatients requiring assistance to move as required.
2. **The Assessment and recording of the details should be undertaken in line with guidance on the front page of the risk assessment booklet.**
3. The assessment should be updated where there is a change in condition and at a minimum every 3 days. It is the responsibility of the nurse caring for a patient at any given time to ensure that the Assessment and Care plan is updated as appropriate. It is the responsibility of ward Sisters/Charge Nurses to ensure their staff are meeting this requirement.
4. All details must be clearly legible.
5. The assessment must be kept within the vicinity of the patient and is accessible by all relevant staff. The form will be retained as a legal record within the patient's notes after discharge.
6. The assessment must be shared with all staff involved in moving the patient e.g. when temporarily transferred to other departments e.g. X-Ray, Theatres.

C. Completing the Document

1. Personal details of will be recorded on the document booklet
2. Identify if the patient has had a fall in the last 12 months or age 65 and over. If the answer is yes please complete falls assessment within the booklet.
3. Indicate whether or not the patient requires assistance to move. If the patient does not require assistance or supervision to move, enter **No**, print name and initials at the bottom of page 2. Do not proceed to complete the Care plan.
4. For patients requiring assistance to move complete the numerical assessment and identify level of risk. **Note; if a bed rail is required complete bed rail risk assessment and enter date that the assessment is carried out.** Enter the numerical scoring of the assessment in the traffic light boxes.
5. The patient handling plan must indicate *method, equipment and number of staff* required for **each handling activity**. If the capability of the patient varies, always

record the method which will reduce the risk to the lowest level possible. If an activity is not applicable to the patient or has not yet been assessed indicate this on the plan. Ensure consent is documented on Page 4.

6. Use the “other” handling activity section for any activities not listed or to reflect variability e.g. transferring when patient is fatigued.
7. When updating the patient handling plan it is not necessary to rewrite the previous “handling activity” entry if it still reflects the current handling status.
8. Enter date, time, and signature for all handling activity entries, even if handling activity is identified as n/a or if identifying that the task has not yet been assessed.
9. When specific bariatric equipment is required for the Plus size patient you will need to complete the grid highlighting specific bariatric equipment. Following this please ensure this patient is referred via SALUS in line with plus size policy.
10. Please Ensure the patient Consent box is completed
11. If assessment indicates complex handling issues and further advice is required, contact the Moving and Handling Team for expert advice, Bleep 81413

D. Outcomes

1. All patients will have the MH risk assessment completed. This will be achieved within 6 hours of admission.
2. All patients requiring assistance to move will have the numerical assessment completed.
3. All patients requiring supervision or assistance to will have a patient handling Care plan completed.
4. ***On transfer to another department this document will accompany the patient.*** On discharge home when care is to be provided by Trust staff or another agency, a summary of handling requirements must be supplied.
5. The Assessment and Care plan will reflect current handling status within a reasonably practicable time frame.
6. The Assessment must be freely available for all relevant staff to be guided on the safest handling method.
7. Staff will be required to refer to this assessment and plan prior to moving the patient.

E. The Traffic Light system

Following completion of the numerical assessment, the patient should be given a colour rating from the traffic light code (red, yellow or green). This illustrates the level of risk. Below is an outline:

Red: High Risk (Score 20+) for example, a dependant patient unable to move themselves.

Yellow: Moderate Risk (11-19) May require assistance of 1 or 2 staff, plus equipment.

Green: Low Risk (1-10) Minimal assistance, supervision or verbal prompts.

A. Standard

A paediatric manual handling assessment must be completed for all patients who are admitted 0 to 18 years old (Manual Handling Operation Regulations 1992). i.e. completion of this form is a statutory requirement.

B. General information regarding assessment

1. The assessment forms part of a wider risk assessment booklet. All patients admitted must have an assessment completed within 6 hours of admission. The assessment may also be used for outpatients requiring assistance to move as required.
2. The assessment and recording of the details should be undertaken by a registered nurse during the initial stages of the patient admission. Any registered health care professional (HCP) can contribute to the patient handling plan as appropriate.
3. The assessment is updated where there is a change in condition and at a minimum every 3 days. It is the responsibility of the HCP caring for a patient at any given time to ensure that the assessment is updated as appropriate. It is the responsibility of ward Sisters/Charge Nurses to ensure their staff are meeting this requirement.
4. All details must be recorded in black ink and must be clearly legible.
5. The assessment must be accessible by all relevant staff. The form will be retained as a legal record within the patient's notes after discharge.
6. The assessment must be shared with all staff involved in moving the patient e.g. when temporarily transferred to other departments e.g. X-Ray, Theatres.

C. Completing the form

1. Personal details of name, weight date of birth and hospital number must be recorded. Location on admission and date of admission must also be recorded.
2. Indicate whether the child development milestones have been reached.
3. Under each activity using the appropriate weight guidance column identify dependence /independence as identified in the document
4. Then using the appropriate columns indicates *Technique, equipment and number of staff* required for each handling activity. If the capability of the patient varies, always record the method which will reduce the risk to the lowest level possible. Ensure consent is documented as identified in the document
5. Use the "other" handling activity section for any activities not listed or to reflect variability e.g. transferring when patient is fatigued.
6. Ensure child history is recorded regarding falling out of bed including identifying if a child has more than 'usual' amount of falls when mobilising.
7. When updating the patient handling plan use the reassessment /documentation of changes. If no change state 'no change'
8. Enter date, time, and signature on the reassessment sheets.
9. TRW.MAH.GUI.1106.1 Paediatric manual Handling mobility assessment guidance Paediatric mobility assessment guidance August 2017 Page 2
10. If assessment indicates complex handling issues and further advice is required, contact the Manual Handling Team for expert advice, Bleep 89748

D. Outcomes

1. All patients will have the risk assessment chart completed. This will be achieved within 6 hours of admission.
2. All patients requiring supervision or assistance to move will have a patient handling plan completed comprehensively using the appropriate column for each activity based on weight.
3. ***On transfer to another department this form will accompany the patient.*** On discharge home when care is to be provided by trust staff or another agency, a summary of handling requirements must be supplied.
4. The assessment will reflect current handling status within a reasonably practicable time frame.
5. The assessment will be freely available for all relevant staff to be guided on the safest handling method.
6. Staff will be required to refer to this assessment prior to moving the patient.

Appendix 5 (Links) for governance only once document approved will be stored separately

- <https://www.hse.gov.uk/pubns/indg383.pdf>
<https://www.hse.gov.uk/pubns/indg478.pdf>
- <http://staffnet.plymouth.nhs.uk/LearningDevelopment/MandatoryTraining/ManualHandling/ManualHandlingTeam.aspx>
- Xcel training documents